Northbridge General Hospital (fictional)

100 Example Avenue, Northbridge - fictional address, telephone 000 0000

Discharge Summary

Department of Respiratory Medicine

PATIENT **Daniel Okafor**	UHID **MG25003**	AGE / SEX **58 years / Male**
REPORT DATE **30 Aug 2026**	REFERENCE NO. **DSC-49347770**	DEPARTMENT **Department of Respiratory Medicine**

ADMITTED **24 Aug 2026**	DISCHARGED **30 Aug 2026**	WARD **Respiratory Ward 6A**

Diagnosis

- Acute infective exacerbation of COPD (GOLD grade 2)
- No ventilatory support required

Presenting Complaint

Five days of increasing breathlessness, cough and purulent sputum with fever. He was unable to speak in full sentences at home and was brought in by ambulance.

Observations

Observation	On admission	At discharge
SpO2 (room air)	88 %	93 %
Respiratory rate	28 /min	18 /min
Heart rate	112 /min	84 /min
Temperature	37.9 C	36.8 C

Examination on Admission

- Using accessory muscles; speaking in short phrases.
- Widespread expiratory wheeze with reduced air entry at both bases.
- No peripheral oedema; jugular venous pressure not raised.
- Alert and oriented; no flapping tremor.

Investigations

Test	Result	Unit	Reference range	Flag
Blood tests on admission				
Haemoglobin	14.9	g/dL	13.0 - 17.0	
White cell count	**14.8**	10^9/L	4.0 - 11.0	H
C-reactive protein	**86**	mg/L	< 5	H
Arterial blood gas on room air				
pH	**7.31**		7.35 - 7.45	L
PaCO2	**58**	mmHg	35 - 45	H
PaO2	**54**	mmHg	80 - 100	L
Bicarbonate	**29**	mmol/L	22 - 28	H

SYNTHETIC DATA — FOR DEMONSTRATION ONLY

Microbiology and Imaging

Sputum culture grew Haemophilus influenzae. Chest X-ray showed hyperinflation without consolidation. Renal function and electrolytes were normal.

Antibiotic	Susceptibility
Amoxicillin	Sensitive
Doxycycline	Sensitive
Clarithromycin	Sensitive
Co-trimoxazole	Resistant

Hospital Course

He received nebulised salbutamol and ipratropium, controlled oxygen titrated to 88-92 %, oral prednisolone 30 mg daily for 5 days and oral doxycycline 100 mg twice daily. Symptoms improved steadily and oxygen was weaned off on day 3.

Day	Date	Progress
1	24 Aug 2026	Admitted via emergency department; 4-hourly nebulisers, oxygen.
2	25 Aug 2026	Wheeze improving; started prednisolone and doxycycline.
3	26 Aug 2026	Oxygen weaned to room air; nebulisers reduced to 6-hourly.
4	27 Aug 2026	Switched to inhalers; walking on the ward with a physiotherapist.
5	28 Aug 2026	Prednisolone course completed; sputum volume reduced.
6	30 Aug 2026	Inhaler technique reviewed; discharged home.

SYNTHETIC DATA — FOR DEMONSTRATION ONLY

Pharmacy Medication Reconciliation

Completed by the ward pharmacist on 25 Aug 2026 from the patient's own account and his general practice record.

Allergies and Adverse Drug Reactions

Penicillin (amoxicillin): urticaria, 2012. No other known allergies.

Medications on Discharge

Medicine	Strength	Dose	Frequency	Remarks
Budesonide/formoterol (Symbicort)	160/4.5 mcg	2 puffs	Twice daily (BD)	Increased from 1 puff twice daily.
Tiotropium (Spiriva)	18 mcg	1 capsule	Once daily	Unchanged.
Salbutamol (Ventolin)	100 mcg/puff	2 puffs	As needed (PRN)	Unchanged.
Doxycycline	100 mg	1 capsule	Twice daily (BD)	Completes 5 days.
Prednisolone	30 mg	-	-	Course completed in hospital.

Social History and Smoking Status

Ex-smoker; 30 pack-years; stopped smoking in 2019. Lives with his daughter. No alcohol excess.

Follow-up

- Respiratory clinic in 6 weeks with repeat spirometry.
- Pulmonary rehabilitation referral sent.
- General practitioner review within 7 days of discharge.
- Return if breathlessness worsens, sputum turns green or fever recurs.

SYNTHETIC DATA — FOR DEMONSTRATION ONLY

COPD Self-management Plan

Reviewed with Mr. Okafor and his daughter by the respiratory nurse specialist.

Zone	How you feel	What to do
Green	Usual breathlessness and cough; sleeping well	Take regular inhalers; keep active; attend rehabilitation.
Amber	More breathless, more sputum or sputum changes colour for two days	Use the reliever as needed and contact the general practice the same day.
Red	Breathless at rest, confusion, blue lips or chest pain	Call the emergency services.

Inhaler Technique Checklist

- Shake the inhaler, breathe out gently, then seal your lips on the mouthpiece.
- Breathe in steadily while pressing once; hold your breath for 10 seconds.
- Wait 30 seconds between puffs; rinse your mouth after the steroid inhaler.
- Tiotropium capsule: pierce once, inhale twice from the same capsule.

Vaccinations

Influenza vaccine given on 02 Oct 2025; pneumococcal vaccine given in 2024.

Electronically signed: **Dr. Fiona Gallagher, Consultant Respiratory Physician** on 30 Aug 2026

*** End of report ***

SYNTHETIC DATA — FOR DEMONSTRATION ONLY